



RESULTS FOR QUERY:

Metastatic Squamous NSCLC 2L Landscape Analysis

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i would like to run the agent for metastatic squamous NSCLC, with an emphasis on 2L. Make sure to evaluate CLND1 targeted therapies in the broader set.

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Therapeutic Area Landscape Analysis: Metastatic Squamous NSCLC Second-Line Treatment Evolution

Key Takeaways

- **Docetaxel remains the entrenched 2L standard** with poor outcomes: median OS of only 9.8-11.8 months, PFS of 3-4 months, and response rates below 25%, creating urgent unmet need for novel therapeutic approaches. 
- **Claudin-1 (CLDN1) emerges as a first-in-class target** with Alentis' ALE.P02 and ALE.P03 ADCs disrupting oncogenic AKT/mTOR pathways and tumor protective barriers while driving cancer stem cells toward differentiated, less invasive phenotypes. 
- **Next-generation bispecifics demonstrate breakthrough survival signals** : TAK-928/IBI363 (PD-1/IL-2 α) achieved 18.2-month median OS with 47.8% 24-month OS rate in IO-resistant patients, while gotistobart (anti-CTLA-4) showed 54% reduction in death risk versus docetaxel.  
- **Broader Claudin family targeting expands** beyond CLDN1 to include CLDN18.2 ADCs (TAK-921/IBI343) and CLDN6 bispecific T-cell engagers (BGB-A455, CTIM-76), capitalizing on surface-exposed tight-junction proteins in malignant transformation with minimal healthy tissue expression. 
- **Multiple pivotal readouts converge in 2026-2027** : Phase III MarsLight-11 (TAK-928), PRESERVE-003 interim data (gotistobart), HLX43 Phase II/III MRCT initiation, and Alentis' Phase I/II safety/efficacy milestones for CLDN1 ADCs will potentially displace chemotherapy standards. 
- **High unmet need persists in genomically-defined subsets** : KEAP1/NFE2L2 mutants (~15% of NSCLC), PIK3CA-altered (~2-4%), and SMARCA4 mutants (~10%) show resistance to standard 2L therapy, presenting high strategic opportunity rating for targeted interventions. 

- **Selectivity engineering becomes critical differentiator** : CTIM-76's asymmetric IgG backbone achieves ~500x selectivity over other claudins to avoid off-target toxicities, contrasting with competitor formats that risk avidity-driven binding to CLDN3/4/9 in healthy tissues. 

Disease Overview & Patient Landscape

Disease Burden & Epidemiology

Lung cancer remains the leading cause of cancer-related mortality globally, posing an immense public health challenge. ^[1] Non-small cell lung cancer (NSCLC) constitutes approximately 85% of all lung cancer cases. ^[2] Squamous cell carcinoma represents roughly 30% of the overall NSCLC population, presenting a highly aggressive, genetically complex, and distinct histology. ^[3]  1 • Press Release  2 • 10K  3 • Bernstein Research

The table below outlines the regional epidemiological burden and demographic profiles across major markets.

Region	Diagnosed Prevalence	Incidence Rate	Patient Population Growth	Key Demographics	Sources
United States	~20,000 addressable metastatic squamous cases annually (calculated from 40,000 US/EU5 pool)	194,999 NSCLC cases in 2026 (estimate)	Declining at ~1.5% annually due to decreased smoking rates	Average age of 70 years; 51% male; over-represented in elderly populations	[4]
Europe (EU5)	~20,000 addressable metastatic squamous cases annually (calculated from 40,000 US/EU5 pool)	~275,000 lung cancer cases in EU4 and UK (2025 estimate)	Flat growth projected with no immediate smoking-driven decline	Highest global smoking prevalence (~24.1%); predominantly male cohorts	[5]
China	N/A	946,700 NSCLC cases in 2024 (reported)	Growing at ~3% CAGR to reach 1,119,300 cases by 2033 (estimate)	40-50% EGFR mutation rate; low female smoking rate but closing gender gap	[6]

In the United States, approximately 55,000 squamous NSCLC patients initiate first-line treatment annually. Roughly 30% of these patients eventually progress to second-line (2L) therapy and become eligible for subsequent systemic regimens. ^[8] In Europe, squamous cell lung cancer accounts for a significant portion of the 484,306 lung cancer incidence cases recorded in 2022. ^[9]  7  8 • Investor Presentation  9 • Investor Presentation

Patient Journey & Treatment Paradigm

The clinical management of metastatic squamous NSCLC is highly standardized but carries a very poor prognosis. ^[10]  10 • TD Cowen

Patients are segmented primarily by histology and the presence of actionable genomic alterations, which are notably rare in the squamous subtype. ^[11]  11 • Bernstein Research

- **First-Line (1L) Standard of Care** : Patients without actionable driver mutations are primarily treated with checkpoint inhibitors (such as pembrolizumab) combined with platinum-doublet chemotherapy. ^[12]  12 • TD Cowen
- **First-Line (1L) Regimen Backbones** : Specifically, standard 1L regimens utilize a combination of carboplatin and either paclitaxel or nab-paclitaxel. ^[13]  13 • Company Publication
- **Second-Line (2L) Transition** : Upon progression on or after 1L immunotherapy and chemotherapy, patient options become severely constrained. ^[14]  14 • Investor Presentation
- **Second-Line (2L) Standard of Care** : Docetaxel (with or without the anti-angiogenic agent ramucirumab) remains the entrenched global standard of care for 2L squamous NSCLC. ^[15]  15 • Investor Presentation
- **Clinical Biomarkers and Patient Segmentation** : Unlike adenocarcinoma, which features a high rate of targetable driver mutations like EGFR or ALK, squamous NSCLC lacks common actionable alterations. ^[17] Instead, patient segmentation in the squamous setting relies heavily on PD-L1 Tumor Proportion Score (TPS) and emerging targets like the Claudin family. ^[16] For instance, over 90% of sqNSCLC patients are positive for PRAME and/or MAGEA4/8, which could support future precision medicine approaches. ^[18]  17 • Bernstein Research  16  18 • ARS

Expert Insights & Physician Perspectives

Thoracic oncologists indicate that 2L treatment choices are heavily influenced by the patient's performance status, age, and previous response to immunotherapy.

- **Frontline Durability Gaps** : According to a thoracic medical oncologist, squamous NSCLC patients historically respond to first-line chemoimmunotherapy, but these responses are frequently less durable, and subsequent second-line options are exceptionally poor after taxane-containing frontline regimens. ^[19]  19 • JMP Securities
- **Clinical Trial Extrapolation Concerns** : Clinicians note that U.S. patient populations are older, with a median age of 70, meaning that clinical trials conducted in younger, highly selected patient cohorts (such as certain China-only trials) may not fully translate to real-world Western practice. ^[20]  20 • Leerink Partners
- **Demographic Nuances** : Oncologists also emphasize that up to 40% of squamous NSCLC in the U.S. occurs in women, who are more likely to harbor driver mutations than their male counterparts. ^[21]  21 • Scotiabank

- **Evolution of Treatment Lines** : In the post-PD-1 era, conventional doublet chemotherapy regimens have generally migrated to the second or third line of metastatic treatment. ^[22]  22 • TD Cowen
- **Development of Novel Backbones** : Biopharma developers expect CTLA-4 inhibitors like gotistobart to provide a survival advantage, with phase III Stage 1 data previously showing a 54% reduction in the risk of death compared to docetaxel in the 2L+ squamous setting. ^[23]  23 • Morgan Stanley Research

The therapeutic landscape of 2L metastatic squamous NSCLC is defined by several profound unmet needs. ^[24]

-  24 • Investor Presentation
- **Limited Efficacy of Current Options** : The current standard of care, docetaxel, delivers a median overall survival (OS) of only 9.8 to 11.8 months in clinical trials, with a progression-free survival (PFS) of just 3 to 4 months. ^[25] ^[26]  25 • Investor Presentation  26 • Bernstein Research
- **Poor Response Rates** : Fewer than 25% of squamous patients respond to 2L chemotherapy-based regimens. ^[27]  27 • Investor Presentation
- **High Toxicity and Tolerability Burden** : Chemotherapy carries substantial systemic toxicities, including severe neutropenia, anemia, neuropathy, and alopecia, which significantly compromise the patient's quality of life. ^[28]  28 • Investor Presentation
- **VEGF Class Concerns** : Historically, targeting angiogenesis via VEGF inhibitors has been highly problematic in squamous histology due to a class-specific risk of severe pulmonary hemorrhage, restricting treatment options. ^[29]  29 • HSBC Global Investment Research
- **Quality of Life Considerations** : Given that metastatic squamous NSCLC predominantly affects elderly patients who have multiple comorbidities, there is an urgent need for chemotherapy-free regimens that prolong survival without exacerbating physical functioning and role functioning limitations. ^[30]  30 • Earnings Presentation

Data current through July 2026

Competitive Landscape & Key Players

Market Structure

The market structure of second-line (2L) metastatic squamous non-small cell lung cancer (sqNSCLC) is highly consolidated around chemotherapy, primarily because of a historical lack of targeted treatment options for this specific histology. ^[31] While immunotherapy has completely transformed first-line (1L) outcomes, standard regimens after progression on chemo-immunotherapy remain anchored to docetaxel and anti-angiogenic combination therapy. ^[32] The established therapies currently competing or historically active in this setting are presented in the table below.  31 • Press Release  32 • Investor Presentation

Company	Key Products	Market Position	Mechanism of Action	Launch Year	Estimated Market Share*	Key Differentiators	Sources
Sanofi / Genentech	Docetaxel (Taxotere)	Entrenched standard of care backbone	Microtubule inhibitor	1996 (US)	Predominant 2L chemotherapy backbone, utilized in the majority of 2L squamous regimens as of H1 2026	Broad real-world utility but high hematological toxicity and severe neutropenia (30-55% Grade 3+)	[33]
Eli Lilly	Cyramza (ramucirumab)	Approved combination agent with docetaxel	VEGFR2 antagonist mAb	2014 (US) / 2016 (EU)	Clinically significant option in post-chemoimmunotherapy settings; exact TRx share in 2L squamous NSCLC is not separately reported in LLY 2025/2026 earnings disclosures	First anti-angiogenic therapy showing survival benefits in 2L NSCLC without histology-restricted exclusions	[34]
Bristol Myers Squibb	Opdivo (nivolumab)	Historically approved 2L immunotherapy	PD-1 inhibitor mAb	2015 (US)	Second-line market share has substantially declined to minority/residual levels as of H1 2026 due to frontline IO migration	Provides long-term survival tail, but clinical utility in 2L is limited to patients who did not receive frontline PD-1 agents	[35]
Merk & Co.	Keytruda (pembrolizumab)	Historically approved 2L immunotherapy	PD-1 inhibitor mAb	2015 (US)	Minimal active 2L TRx share as of H1 2026 due to extensive (~95%+) penetration of Keytruda-based frontline regimens	Exceptional frontline standard, which structurally depleted the eligible patient pool for 2L PD-1 monotherapy	[36]

Strategic Positioning & Platform Advantages

Biopharmaceutical developers in the squamous NSCLC landscape are increasingly forced to differentiate their approaches due to the strict limitations of standard chemotherapy.^[37] Because squamous tumors frequently lack standard actionable driver mutations (such as EGFR, ALK, or ROS1), standard targeted small-molecule tyrosine kinase inhibitors (TKIs) are rarely applicable to this cohort.^[38]

 38 • Bernstein Research

 37 • Investor Presentation

To address this gap, companies are prioritizing three primary therapeutic platforms: next-generation immunotherapies, antibody-drug conjugates (ADCs), and bispecific platforms.^[39]

 39 • H.C. Wainwright

- **Next-Generation Immunotherapies (CTLA-4 and Tumor Vaccines)** : Companies like BioNTech are seeking to upgrade the immunotherapy backbone. ^[41] BioNTech is leveraging gotistobart (anti-CTLA-4), which is designed to selectively deplete regulatory T cells (Tregs) and enhance anti-tumor activity in cold or IO-refractory microenvironments. ^[40]  41 • Huatai Financial Holdings (HK)  40 • Investor Presentation
- **Next-Generation Immunotherapies (CTLA-4 and Tumor Vaccines) (cont.)**: This approach aims to restore immune recognition in patients who have already progressed on standard anti-PD-1 therapies. ^[42]  42 • Bernstein Research
- **Antibody-Drug Conjugates (ADCs)** : The use of target-specific ADCs represents a strategic shift toward replacing conventional chemotherapy with cytotoxic agents delivered directly to the tumor site. ^[43]  43 • Huatai Financial Holdings (HK)
- **Antibody-Drug Conjugates (ADCs) (cont.)**: While TROP2-targeted ADCs have shown promising activity, clinical differentiation relies on target selectivity and managing systemic toxicities, as seen in the development of sacituzumab tirumotecan (sac-TMT). ^[44]  44 • Bank of America Global Research
- **Bispecific Platforms (PD-1 x VEGF and T-Cell Engagers)** : Bispecific antibodies dually targeting PD-1 and VEGF (such as ivonescimab) aim to capitalize on the synergistic anti-tumor effects of combining immune checkpoint blockade with anti-angiogenesis. ^[45] This platform holds a distinct safety advantage, particularly in squamous tumors where older anti-VEGF class agents were historically avoided due to severe pulmonary hemorrhage risks. ^[46]  45  46 • TD Cowen
- **Bispecific Platforms (PD-1 x VEGF and T-Cell Engagers) (cont.)**: Additionally, bispecific T-cell engagers (TCEs) are being deployed to bypass conventional antigen presentation and directly recruit T cells to squamous-associated targets.

Recent M&A and Clinical Collaboration Agreements

Strategic transactions in the broader oncology space highlight the intense interest in securing advanced lung cancer assets, particularly bispecific antibodies, ADCs, and highly selective targeted platforms. ^[47]

-  47 • H.C. Wainwright
- **GSK and Nuvalent Acquisition** : In mid-2026, GSK announced a strategic acquisition of Nuvalent in a transaction worth up to a 40% premium, aiming to rapidly build out its lung cancer portfolio and global commercial presence. ^[48] This transaction secures Nuvalent's potential best-in-class ROS1 and ALK inhibitors (zidesamtinib and neladalkib), which are set for US FDA action in late 2026. ^[49]  48  49 • Bernstein Research
- **AbbVie and RemeGen Licensing Deal** : In January 2026, AbbVie entered into a licensing agreement to acquire global rights (excluding China) to RemeGen's PD-1/VEGF bispecific antibody candidate, RC148. ^[50] The deal features a \$650 million upfront payment, up to \$4.95 billion in aggregate milestones, and tiered double-digit

- royalties. ^[51] AbbVie intends to evaluate RC148 in combination with its internal pipeline, including the c-MET ADC telisotuzumab adagranodermin (Temab-A) in NSCLC. ^[52]  50  51  52 • Leerink Partners
- **Innovent and Eli Lilly Oncology Partnership** : In February 2026, Innovent and Eli Lilly expanded their strategic relationship with a major preclinical collaboration in oncology and immunology, triggering a \$350 million upfront payment and up to \$8.5 billion in potential milestones. ^[53] This agreement enables Innovent to lead R&D through proof-of-concept, while leveraging Lilly's massive global commercialization and clinical execution engines for subsequent development. ^[54]  53  54 • UBS Research
 - **Coherus Oncology and Janssen Collaboration** : In February 2026, Coherus Oncology entered into a clinical supply agreement with Janssen Research & Development to evaluate its anti-CCR8 antibody, tagmokitug, in combination with Janssen's T-cell engaging bispecific antibody, pasritamig, in a Phase 1b clinical trial. ^[55]  55 • ARS

Data current through July 2026

Clinical Pipeline Analysis

Pipeline Overview

The clinical pipeline for second-line (**2L**) metastatic squamous non-small cell lung cancer (**sqNSCLC**) is characterized by a strategic pivot away from non-selective chemotherapies toward highly targeted platforms, with the tight-junction **Claudin (CLDN) protein family** and next-generation bispecifics emerging as the primary frontiers of innovation. ^[56] Historically, squamous tumors have lacked the targetable driver mutations common in adenocarcinoma, but the discovery of tumor-associated exposure of proteins like **CLDN1** , **CLDN18.2** , and **CLDN6** has opened novel therapeutic windows. ^[57]  56 • Press Release  57 • William Blair Research

The table below outlines the late-stage and biologically significant emerging pipeline candidates currently in clinical development for **2L sqNSCLC** and related squamous histologies.

Drug/ Candi date Name	Com pany	Mechani sm of Action	Curre nt Phase	Primary Indication	Expected Data Readout/ Timeline	NC T Nu mb er	Expe dited Statu s**	Key Differentiators	S o ur ce s
TAK-928 / IBI363	Take da / Innov ent Biolo gics	PD-1 / IL-2 α -biased bispecific antibody fusion protein	Phase III (Mars Light-11)	2L Squamous NSCLC	sq cohort final PFS and interim OS expected in 2H 2026	NC T07 217 301	FDA Fast Track Designation	Dual mechanism combining immune checkpoint blockade and selective IL-2 α -biased cytokine activation; shows robust OS tail in IO-resistant sqNSCLC	[58]
Gotistobart (BNT316 / ONC-392)	BioN Tech / Onco C4	Next-generation anti-CTLA-4 mAb	Phase III (PRESERVE-003)	2L+ Squamous NSCLC	Interim Phase III data expected in 2026	NC T05 671 510	None identified	Selectively depletes regulatory T cells (Tregs) in cold tumor microenvironments, demonstrating a 54% reduction in risk of death	[59]
HLX43	Shan ghai Henli us Biote ch	PD-L1-targeted antibody-drug conjugate (ADC)	Phase II/III (HLX43-NSCLC302)	Previously treated advanced/metastatic sqNSCLC	Phase II/III MRCT initiated in May 2026	No ne ide ntifi ed	None identified	Potentially best-in-class ADC integrating immune checkpoint blockade with payload-mediated cytotoxicity	[60]
ALE.P02	Alenti s Thera peuti cs	CLDN1-targeted ADC (tubulin payload)	Phase I/II	CLDN1+ advanced/metastatic squamous solid tumors	Clinical data readouts expected in 2026/2027	No ne ide ntifi ed	None identified	First-in-class ADC targeting non-junctional Claudin-1 to open tumor protective barriers and deliver a tubulin-inhibiting payload	[61]

Drug/ Candidate Name	Company	Mechanism of Action	Current Phase	Primary Indication	Expected Data Readout/ Timeline	NC T Number	Expedited Status**	Key Differentiators	Sources
ALE.P03	Alentis Therapeutics	CLDN1-targeted ADC (topoisomerase I payload)	Phase I/II	CLDN1+ advanced/metastatic solid tumors	Clinical data readouts expected in 2026/2027	No ne identified	None identified	Leverages a topoisomerase I inhibitor payload to reprogram cancer cell states and drive stem-like tumor cells toward differentiation	[62]
TAK-921 / IBI343	Take da / Innovent Biologics	CLDN18.2-targeted ADC	Phase I/II	Advanced solid tumors (including sqNSCLC subsets)	Clinical updates ongoing	No ne identified	None identified	Site-specific conjugation targeting Claudin 18.2 with a topoisomerase I payload, minimizing off-target toxicities	[63]
BGB-A455	BeOn e Medicines	Claudin6 x CD3 bispecific antibody	Phase I	Solid tumors	Clinical updates ongoing	No ne identified	None identified	Directs CD3+ T cells to CLDN6-expressing cancer cells to induce localized tumor cell death	[64]

Pipeline Dynamics & Target Modalities

- Claudin-1 (CLDN1) Targeted Therapies** : Alentis Therapeutics is pioneering the targeting of **CLDN1** , a tight-junction protein that is frequently overexpressed and mis-localized in squamous cancers. [66] Mechanistically, inhibiting **CLDN1** disrupts key oncogenic cascades like the AKT/mTOR pathway and reprogrammable tumor cell plasticity, driving stem-like cancer cells toward a more differentiated, less invasive epithelial phenotype. [65] By targeting non-junctional, tumor-exposed **CLDN1** , Alentis' ADCs (**ALE.P02** and **ALE.P03**) aim to open up the protective physical barrier surrounding tumors, preventing them from evading the host immune system while directly delivering highly cytotoxic payloads. [67]
 - 66
 - 67 • Morgan Stanley Research
 - 65 • PubMed
- Broader Claudin Family Targets** : Beyond **CLDN1** , developers are exploiting other members of the Claudin family, including **CLDN18.2** and **CLDN6** , which are normally sequestered in healthy tight junctions but become surface-exposed during malignant transformation. [69] While **CLDN18.2** is a validated target in gastric cancers, its ectopic expression in subsets of lung cancer has prompted the development of ADCs like **TAK-921/IBI343** to bypass standard chemo-resistance. [68]
 - 69 • William Blair Research
 - 68 • William Blair Research

- **Broader Claudin Family Targets (cont.):** Similarly, **CLDN6** is being actively pursued via bispecific T-cell engagers (TCEs) such as **BGB-A455** and **CTIM-76**, capitalizing on the antigen's near-complete absence in healthy adult tissue to avoid off-target toxicities. ^[70]  70 • RBC Capital Market Research
- **Bispecific and Fusion Protein Platforms** : The integration of dual-targeting mechanisms represents a major design trend, as highlighted by Takeda and Innovent's **TAK-928/IBI363**, a PD-1/IL-2 α -biased bispecific fusion protein. ^[72] This platform simultaneously blocks the PD-1 pathway while selectively activating IL-2 signaling on tumor-infiltrating lymphocytes, avoiding the systemic toxicities historically associated with wild-type IL-2 therapies. ^[71]  72 • INTRM  71 • Investor Presentation

Combination & Biomarker-Driven Strategies

The therapeutic approach in the **2L** setting is increasingly guided by sophisticated biomarker segmentation and combination rationales to overcome immune-cold microenvironments.

- **Biomarker-Driven Selection** : Rather than treating all-comer cohorts, next-generation programs are utilizing stringent patient enrichment strategies. ^[73] For example, Alentis Therapeutics' clinical trials specifically require prospective screening for **CLDN1+** expression in squamous solid tumors. ^[74] Similarly, other emerging programs utilize optimized immunohistochemistry (IHC) cutoffs to identify patients most likely to derive clinical benefit from Claudin-directed therapies. ^[75]  73  75 • Leerink Partners  74 • ClinicalTrials.gov
- **Synergistic Immuno-Oncology Combinations** : Developers are actively pairing novel targeted agents with standard checkpoint inhibitors to establish deep, durable responses. ^[77] Alentis is evaluating **ALE.P02** both as a monotherapy and in combination with the anti-PD-1 antibody pembrolizumab in **CLDN1+** squamous tumors, seeking to turn immunologically "cold" squamous microenvironments "hot." ^[76]  76 • EU Clinical Trials (CTIS)  77 • H.C. Wainwright
- **TCER Dual-Targeting Multiplexing** : Immatics is advancing an innovative "off-the-shelf" T-cell receptor (TCR) bispecific (TCER) strategy. ^[78] The company is preparing to evaluate the combination of **IMA402** (targeting PRAME) and **IMA401** (targeting MAGEA4/8) in **sqNSCLC**. ^[80] Because over **90%** of **sqNSCLC** patients express at least one of these targets, and approximately **60%** are double-positive, this multiplexed bispecific approach is designed to prevent tumor antigen escape and maximize anti-tumor depth. ^[79]  78  80 • ARS  79 • TD Cowen

Development Velocity & Near-Term Catalysts

The race to displace docetaxel as the **2L sqNSCLC** standard of care is driving rapid clinical execution, with several high-impact readouts scheduled over the next **12 to 18 months**.

- **Rapidly Advancing Registrational Programs** : Takeda and Innovent are rapidly advancing the global Phase III **MarsLight-11** trial (NCT07217301) comparing **TAK-928/IBI363** directly against docetaxel in patients who progressed on prior chemo-immunotherapy. ^[82] At **ASCO 2026**, updated Phase I/II data for **IBI363** in immunotherapy-resistant squamous NSCLC demonstrated a median overall survival (OS) of **18.2 months** and

a **24-month OS rate of 47.8%** in the **3 mg/kg Q3W** cohort, representing a major survival tail improvement over historical chemotherapy benchmarks. ^[81] [82 • Earnings Presentation](#) [81 • Press Release](#)

- **BioNTech's gotistobart Pivotal Milestones** : BioNTech and OncoC4's Phase III **PRESERVE-003** trial (NCT05671510) is currently evaluating gotistobart in patients with squamous NSCLC. ^[84] Interim Phase III data is expected to read out in **2026** , with prior non-pivotal data showing a 12-month OS of **63%** versus **30%** for docetaxel. ^[83] Management has indicated that positive interim results could serve as the foundation for an accelerated approval filing with the FDA. ^[85] [84 • Press Release](#) [83](#) [85 • Canaccord Genuity](#)
- **Emerging ADC Milestones** : Shanghai Henlius Biotech is accelerating the international Phase II/III development of its PD-L1-targeted ADC, **HLX43** , with the first patient dosed in China in **May 2026** as part of a multi-regional clinical trial (MRCT) designed to challenge standard docetaxel regimens. ^[86] Meanwhile, Alentis Therapeutics is well-capitalized with cash runway extending into **2027** , supporting multiple Phase I/II safety, tolerability, and initial efficacy readouts for both **ALE.P02** and **ALE.P03** throughout **2026** and **2027** . ^[87] [86 • Press Release](#) [87 • Morgan Stanley Research](#)

Emerging Opportunities & White Space

Underserved Patient Populations

In second-line (2L) metastatic squamous non-small cell lung cancer (sqNSCLC), several molecularly and clinically defined patient subsets remain highly underserved due to the limitations of standard platinum-doublet and checkpoint inhibitor therapies. Standard 2L treatments such as docetaxel offer marginal efficacy, leaving substantial space for targeted interventions in genomic sub-populations.

Patient Segment	Current Treatment Gaps	Estimated Population Size	Level of Competitive Activity	Strategic Opportunity Rating	Sources
KEAP1 / NFE2L2 (NRF2) Mutants	Shorter time to treatment failure, poor overall survival (OS), and resistance to standard 2L chemotherapy and immunotherapy due to metabolic glutamine dependency.	~15% of NSCLC cases (calculated from ~11.3% KEAP1 and ~3.5% NFE2L2 mutually exclusive rates).	Moderate; some historical trial terminations (e.g., KEAPSAKE, MGY825), with ongoing development of sapanisertib.	High	[88]
PIK3CA Altered (Mutated / Amplified)	Predominantly smokers presenting with advanced disease and high rate of concomitant alterations (e.g., KRAS) with poor therapeutic options.	~2% to 4% of NSCLC cases (or ~18% when clustered with NFE2L2/KEAP1 in the broad PI3K/AKT/mTOR pathway).	Low to Moderate; being evaluated in multi-node blockade combination regimens such as the PIKTOR study.	High	[89] [90] [91]
SMARCA4 (BRG1) Mutants	Poor prognosis with significantly shorter OS (~8 months vs. ~15 months wild-type in 1L) and poor response to standard 2L docetaxel.	Up to 10% of NSCLC patients.	Low; pioneering development of selective SMARCA2 synthetic lethal inhibitors (such as FHD-909).	High	[92]
Claudin-Low or Negative Relapsed / Refractory	Emerging Claudin-targeted therapies (e.g., ALE.P02, CTIM-76) require high target expression, leaving negative patients with no options.	~50% to 60% of squamous NSCLC patients (calculated based on ~42% CLDN6-positivity rates).	Low; most clinical developers focus strictly on biomarker-positive patient cohorts.	Medium	[93]

Technical & Innovation Gaps

Novel Mechanisms of Action & Claudin Selectivity Profiles

Targeting Claudin proteins in squamous tumors requires extreme selectivity because of the high structural conservation within the tight-junction gene family.^[94] The primary development hurdle historically for Claudin-6 (CLDN6) targeted therapies has been binding domain homology with CLDN3, CLDN4, and CLDN9, which are

enriched in pancreatic, liver, and ear tissue, creating a high risk of off-target toxicities such as elevated liver enzymes. ^[95]  94 • 10K  95 • H.C. Wainwright

According to reports from H.C. Wainwright, Context Therapeutics' CTIM-76 addresses this challenge by incorporating an asymmetric, Fc-silent, IgG backbone with a highly selective CLDN6 Fab and a full-affinity CD3 arm to achieve ~500x selectivity over other claudins, avoiding the off-target liabilities of competitor formats. ^[96] In contrast, reports from H.C. Wainwright suggest that Xencor's competitor asset, XmAb541, utilizes a bivalent 2+1 format with a detuned CD3 arm that may trigger avidity-driven binding to CLDN3, 4, and 9 at higher doses. ^[97]  96 • H.C. Wainwright  97 • H.C. Wainwright

Additionally, targeting tumor-associated antigens through T-cell receptor (TCR) bispecifics (TCER) offers a mechanism to bypass human leukocyte antigen (HLA) downregulation. ^[98] Immatics is advancing dual targeting in sqNSCLC by combining its TCER candidates, IMA401 (targeting MAGEA4/8) and IMA402 (targeting PRAME), to cover over 90% of the squamous patient population and address potential tumor antigen escape. ^[99]  98 • ARS  99 • TD Cowen

Advanced Drug Delivery Formulations

To improve the therapeutic index of cytotoxic or immunomodulatory payloads, developers are innovating localized and targeted delivery systems:

- **Systemic Lipid-Based Nanoparticles** : Genprex's REQORSA (quaratusugene ozeplasmid) utilizes the non-viral ONCOPREX delivery system to encapsulate the TUSC2 tumor suppressor gene in lipid-based nanoparticles. ^[100] These positively charged lipoplexes exploit the negative charge of hyper-metabolic cancer cells to deliver the payload systemically while sparing healthy tissue. ^[101]  100  101 • ARS
- **Bronchoscopic Injection** : Candel Therapeutics is utilizing outpatient robotic bronchoscopy and transbronchial needle injection (TBNI) to deliver its replication-defective adenoviral vector, aglatimagene, directly into localized lung lesions. ^[102] This local administration avoids systemic exposure and toxicities, converting immunologically cold refractory tumors into hot responsive ones. ^[103]  102  103 • Investor Presentation
- **Inhaled Formulations** : Krystal Biotech is evaluating an inhaled formulation of its gene therapy vector, KB707, which delivers IL-12 and IL-2 cytokines directly to the tumor microenvironment (TME) via inhalation, showing encouraging local responses in target lung lesions. ^[104]  104 • H.C. Wainwright

Companion Diagnostics & Biomarker Strategies

The high rate of tissue insufficiency for biomarker testing—reported to affect approximately 50% of advanced NSCLC patients—presents a major bottleneck. ^[105] This tissue deficit has driven the commercial and clinical adoption of rapid, blood-based diagnostic and monitoring technologies:  105 • H.C. Wainwright

- **Rapid Blood-Based Profiling** : According to reports from H.C. Wainwright, Biodesix's IQLung testing strategy combines GeneStrat NGS (a 52-gene tumor profiling panel) and VeriStrat (an 8-protein proteomic immune

RESULT FOR QUERY

profiling assay) to deliver comprehensive molecular information from a single blood draw within three business days. ^[106]  106 • H.C. Wainwright

- **Treatment Monitoring** : Biovica's DiviTum TKa measures circulating thymidine kinase activity (TKa) via a simple blood draw, providing a highly sensitive pharmacodynamic tool that serves as a continuous risk marker for treatment response and disease progression. ^[107]  107 • ARS
- **Early Screening Integration** : Canaccord Genuity reports that PrognomiQ has simplified its multi-omic diagnostic assay into the blood-based ProVue Lung Laboratory Developed Test (LDT), which utilizes an 11-protein panel to optimize sensitivity for early lung cancer detection. ^[108]  108 • Canaccord Genuity

Geographic & Regulatory Optimization

Maximizing development velocity in 2L squamous NSCLC requires strategic navigation of evolving regulatory frameworks across major global territories.

- **Expedited Trial Designs** : While the FDA has historically tightened approvals for oncology applications by favoring randomized data over single-arm studies, accelerated pathways based on single-arm trials and surrogate endpoints remain accessible for high unmet needs. ^[109]  109 • H.C. Wainwright
- **Expedited Trial Designs (cont.)**: For instance, according to Morgan Stanley, Shanghai Henlius Biotech is actively negotiating with the FDA to pursue a single-arm trial design for the US approval of its PD-L1-targeted ADC, HLX43, in late-line squamous NSCLC. ^[110]  110 • Morgan Stanley Research
- **Project Optimus Dose Optimization** : The Oncology Center of Excellence's (OCE) Project Optimus initiative requires drug developers to thoroughly evaluate and justify dose selection based on safety and tolerability rather than relying solely on traditional maximum tolerated dose (MTD) designs. ^[112] ^[113] Under this framework, companies are integrating validated pharmacodynamic biomarkers, such as Biovica's TKa assay, to support formal dose-optimization plans during Phase I/II development, minimizing downstream regulatory delay. ^[111]  112 • ARS  113 • 10Q  111 • ARS

Data current through July 2026

Catalyst Calendar & Key Upcoming Events

The second-line (2L) metastatic squamous non-small cell lung cancer (sqNSCLC) landscape is entering a crucial transition period, with clinical catalysts over the next 12 to 18 months poised to challenge the long-standing chemotherapy standard. Historically, drug developers have struggled to deliver targeted treatments for squamous histologies due to the lack of targetable genomic alterations. However, the focus has shifted toward highly selective novel pathways, particularly next-generation immunotherapies, antibody-drug conjugates (ADCs), and bispecific platforms targeting tight-junction proteins. ^[114] These near-term events are expected to provide definitive clinical proof-of-concept data for several first-in-class assets. ^[115]  114 • ARS

 115 • Morgan Stanley Research

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The table below outlines the critical clinical and regulatory milestones scheduled across the metastatic squamous NSCLC space over the next 12 to 18 months.

Event Type	Company/Asset	Expected Timing	Potential Impact	Strategic Significance	Sources
Phase III Trial Readout	BioNTech / OncoC4Gotistobart (BNT316 / ONC-392)	2026 (Confidence: High)	Replaces standard docetaxel with a next-generation CTLA-4 monoclonal antibody in 2L+ squamous NSCLC patients who progressed on prior immunotherapy.	Validates selective regulatory T-cell (Treg) depletion in the tumor microenvironment and establishes a regulatory path for accelerated FDA approval.	[116]
Regulatory Decision (PDUFA)	Summit Therapeutics / Akesolvonescimab (AK112)	November 14, 2026 (Confidence: High)	Introduces the first-in-class PD-1 x VEGF bispecific antibody to the Western market for EGFR-mutated, locally advanced or metastatic non-squamous NSCLC.	Establishes commercial proof-of-concept for the dual anti-angiogenesis and immune checkpoint blockade mechanism of action.	[117]
Phase III Trial Readout	AstraZeneca / Daiichi SankyoDatopotamab deruxtecan (Datroway) - AVANZAR Trial	2H 2026 (Confidence: Medium - subject to event-driven completion)	Compares frontline triplet therapy (Datroway + Imfinzi + chemo) against pembrolizumab plus chemotherapy in all-comer and TROP2-high 1L NSCLC.	Shifting the TROP2 ADC into first-line combinations will restructure the downstream 2L patient pool and clinical sequencing options.	[118]
Phase I/II Clinical Readout	Alentis TherapeuticsAL E.P02	2026/2027 (Confidence: High)	Provides the first clinical safety, pharmacokinetics, and initial antitumor efficacy signals in patients with CLDN1+ squamous solid tumors.	Validates the therapeutic concept of targeting tumor-exposed non-junctional Claudin-1, bypassing normal tissue tight-junction toxicity.	[119]
Phase I Trial Initiation	ImmaticsIMA401 + IMA402 combination	Mid-2026 (Confidence: High)	Initiates the first clinical trial evaluating a dual-targeted TCR-bispecific combination therapy in squamous NSCLC.	Evaluates whether co-targeting MAGEA4/8 and PRAME can prevent tumor antigen escape and enhance response depth in immune-cold squamous tumors.	[120]

Event Type	Company/Asset	Expected Timing	Potential Impact	Strategic Significance	Sources
Phase I Dose Expansion	Context TherapeuticsCT IM-76	2H 2026 (Confidence: High)	Expands dosing cohorts to evaluate weekly (QW) versus every-three-week (Q3W) administration of the CLDN6 x CD3 bispecific in solid tumors.	Evaluates systemic tolerability, cytokine release syndrome (CRS) profiles, and therapeutic window for targeting oncofetal tight-junction proteins.	[121]

Strategic Watch List

Biopharmaceutical executives, clinical development teams, and commercial leads should closely monitor the following critical competitive dynamics and strategic questions over the next 12 to 18 months:

- Gotistobart Accelerated Approval Path** : BioNTech and OncoC4 plan to monitor whether Phase III Stage 2 interim results for gotistobart in 2L+ squamous NSCLC can replicate the impressive survival benefit from Stage 1. [122] Successful replication would support a regulatory pathway for accelerated approval with the US FDA. [123]  122  123 • Morgan Stanley Research
- Takeda's Global Expansion of TAK-928** : Takeda and Innovent's progress with the Phase III MarsLight-11 trial (NCT07217301) comparing TAK-928/IBI363 against docetaxel remains a primary competitive focus. [124] Investors are closely tracking whether the strong Phase I/II overall survival signal (mOS of 18.2 months) can be maintained in a larger global multi-regional clinical trial. [126] [125]  124 • ClinicalTrials.gov  126 • Morgan Stanley Research

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Takeda clinical pipeline showing TAK-928 for 2L squamous NSCLC and Claudin-targeting assets

Condition	Squamous Non-small Cell Lung Cancer
Intervention/Treatment	BIOLOGICAL: Intismeran Autogene BIOLOGICAL: Pembrolizumab DRUG: Carboplatin DRUG: Paclitaxel DRUG: Nab-paclitaxel OTHER: Placebo
Phase	Phase 2
Study Type	Interventional
Lead Sponsor	Merck Sharp & Dohme LLC
Status	Recruiting
Enrollment	Anticipated: 180 participants
Posted	October 28, 2025
Updated	June 26, 2026
Primary Completion	July 2, 2029 (Anticipated)
Study Completion	May 6, 2031 (Anticipated)
Study Identifier	NCT07221474 Change History

- Claudin-1 ADC Clinical Validation** : Alentis Therapeutics is gearing up for clinical readouts of its leading CLDN1-targeted ADCs, ALE.P02 and ALE.P03, which are currently enrolling in Phase I/II solid tumor trials. ^[128] The clinical community is watching to see if targeting exposed tumor-associated CLDN1 will avoid the systemic toxicities seen with other tight-junction therapies. ^[129]  128 • Press Release  129 • Press Release
- Selectivity Profiles of CLDN6 T-Cell Engagers** : The dose-escalation phase of Context Therapeutics' CTIM-76 will complete in late 2026, transitioning into Q3W dosing evaluation. ^[130] Competitor programs like Xencor's XmAb541 are also under close monitoring to see if the bivalent 2+1 format can deliver sufficient selectivity over other Claudin proteins. ^[131]  130 • William Blair Research  131 • ARS
- TCR Bispecific Combinations in sqNSCLC** : Immatics' upcoming mid-2026 trial initiation of the IMA401 and IMA402 combination represents a critical step for TCR-bispecific development in sqNSCLC. ^[132] The clinical outcome of this trial will determine whether targeting both MAGEA4/8 and PRAME can overcome tumor antigen escape in squamous histologies. ^[133]  132  133 • ARS

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Strategic Implications & Recommendations

The rapidly evolving therapeutic landscape of second-line (2L) metastatic squamous non-small cell lung cancer (sqNSCLC) demands highly coordinated strategies across clinical development, commercialization, and business development (BD&L) functions.

As next-generation bispecifics and Claudin-targeted therapies move toward registrational milestones, traditional treatment backbones will be displaced.

To maintain competitive advantage, developers must align trial designs, companion diagnostics, and partnership strategies with emerging clinical realities. ^[134]  134 • H.C. Wainwright

The table below outlines actionable recommendations across the primary strategic domains to guide portfolio decision-making.

Strategic Domain	Key Finding	Implication	Recommended Action	Sources
Competitive Positioning	Next-generation CTLA-4 inhibitors (gotistobart) and PD-1/VEGF bispecifics show strong overall survival outcomes compared to standard docetaxel.	Chemotherapy-based regimens will be displaced, but late-stage market entrants face a highly competitive landscape with shrinking market share.	Focus positioning on chemo-free combinations or highly selective, biomarker-guided cohorts to capture market share.	[135]
Development Strategy	Claudin-targeted pipelines (CLDN1, CLDN6, CLDN18.2) require strict patient enrichment, but tissue testing is hindered by high rates (~50%) of tissue insufficiency.	Relying solely on conventional tissue-based next-generation sequencing (NGS) results in high screening failure rates and trial delays.	Integrate rapid, blood-based diagnostic assays and circulating tumor DNA (ctDNA) monitoring to accelerate patient selection and trial enrollment.	[136]
Partnership/M&A	Big Pharma is actively executing multi-billion dollar acquisitions and licensing deals to secure next-generation cell therapy, bispecific, and antibody-drug conjugate (ADC) platforms.	Valuation premiums are concentrated around platform technologies that can generate multiple oncology products.	Pursue strategic licensing deals with clinical-stage targets (such as Context's CLDN6 T-cell engagers or Alentis' CLDN1 ADCs) to establish early platform leadership.	[137]
Risk Assessment	Key pivotal trials for leading bispecific platforms have been conducted in younger, highly selected Chinese patient cohorts.	Western regulatory agencies and payers will challenge the transferability of clinical trial efficacy and safety data to real-world Western populations.	Execute multi-regional clinical trials (MRCTs) that proactively enroll older, demographically representative Western cohorts to mitigate regulatory approval risk.	[138]

Actionable Guidance for Clinical Development

When designing clinical trials in the 2L squamous NSCLC setting, drug developers must prioritize tolerability and alternative delivery systems. ^[139] Chemo-free backbones are highly preferred by clinicians due to the cumulative physical deterioration of patients post-frontline platinum-doublet therapy. ^[140] To mitigate the systemic side effects and cytokine release syndrome (CRS) commonly associated with T-cell engaging (TCE) bispecific antibodies, developers should utilize subcutaneous administration routes. ^[141] For example, Astellas' subcutaneous dosing of its CLDN18.2 T-cell engager, ASP2138, has shown a favorable tolerability profile with minimal high-grade CRS in early clinical evaluations. ^[142]  139  140 • Expert Call  141  142 • IRPRESS

Clinical development teams must also design dosing schedules that prevent rapid T-cell exhaustion. ^[143] Pharmacokinetic simulations suggest that less frequent dosing intervals (such as every two or three weeks) maintain drug concentrations above the minimum effective threshold while allowing a necessary pharmacological washout period for T-cell recovery and expansion. ^[144] Furthermore, clinical trials must be demographically representative; over 40% of squamous NSCLC patients in Western territories are over the age of 65, meaning that trial designs must avoid younger-age selection bias to ensure regulatory transferability to the FDA and EMA. ^[145]  143  144 • Guggenheim Securities  145 • Expert Call

Biomarker-Driven Patient Selection

To maximize objective response rates and mitigate off-target toxicities, clinical development strategies must rely on precise biomarker enrichment and dual-targeting strategies. ^[146]  146 • Guggenheim Securities

- **Multiplexed TCR Bispecific Selection** : To address the risk of tumor antigen escape, developers should pursue multiplexed T-cell receptor (TCR) bispecific (TCER) combinations. ^[147] Immatics is exploring the combination of IMA402 (targeting PRAME) and IMA401 (targeting MAGEA4/8); because over 90% of squamous NSCLC patients express at least one of these targets, this approach provides nearly complete population coverage and prevents clonal escape. ^[148]  147  148 • ARS
- **Claudin-1 Enriched Screening** : For programs targeting Claudin-1 (CLDN1), prospective clinical trials must integrate immunohistochemistry (IHC) screening protocols to select patients with tumor-exposed, non-junctional CLDN1 expression, as pioneered by Alentis Therapeutics. ^[149]  149 • Morgan Stanley Research
- **Selectivity-Driven formats for Claudin-6** : Preclinical and clinical characterization of CLDN6-targeted therapies must utilize binding architectures that demonstrate at least 500x selectivity over closely related proteins CLDN3, CLDN4, and CLDN9 to prevent severe, target-mediated liver and tissue toxicities. ^[150]  150 • H.C. Wainwright
- **Longitudinal Circulating Biomarker Monitoring** : To continuously evaluate treatment efficacy and early disease progression, trials should utilize blood-based monitoring tools. ^[151] Biovica's DiviTum TKa assay, which measures circulating thymidine kinase activity, has demonstrated strong prognostic value and can detect early treatment-specific biological responses from simple blood draws. ^[152]  151  152 • Redeye

Business Development & Partnership Opportunities

As competition intensifies in the 2L squamous NSCLC space, biopharmaceutical companies should actively target partnerships with clinical-stage platform developers to expand their oncology pipelines. ^[153]

 153 • Canaccord Genuity

- **Context Therapeutics** : Context Therapeutics represents an attractive collaboration target due to its selective CLDN6 x CD3 bispecific antibody CTIM-76, which is strategically positioned for the large cohort of post-ADC platinum-resistant patients. ^[154]  154 • H.C. Wainwright
- **Alentis Therapeutics** : With its Series D funding of \$181.4 million secured in late 2024, Alentis is highly capitalized to advance its first-in-class CLDN1-targeted ADCs (ALE.P02 and ALE.P03) through Phase I/II readouts, offering high-value licensing opportunities in the squamous-origin solid tumor space. ^[155]  155 • KeyBanc
- **Tango Therapeutics** : Partnering with Tango Therapeutics to combine vopimetostat (a PRMT5 inhibitor) with pan-RAS inhibitors represents a high-potential combination strategy that could provide a distinct clinical edge in the crowded KRAS-mutant squamous lung cancer market. ^[156]  156 • Canaccord Genuity

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